Digi Pharm

R1 POC – GAP Analysis

Cluster R1 Data Gap Analysis Report

Finance Data Gaps

- **Cost Correlation between Facility Reported Data and PLICS Data**: The current system does not allow for a comprehensive correlation between costs reported by the healthcare facility and the Patient Level Information and Costing Systems (PLICS) data. We need to bridge this gap to compare and validate the costs attributed to various areas, including medicines, consumables, staff, and others.

- **Periodic Cost Analysis**: There is a gap in comparing and analysing costs reported in the SAG report with the corresponding quarter's PLICS data. Implementing this check will allow for timely validation and discrepancy resolution.

Finance Data Quality Checks

- **Data Validation Checks:** An automatic validation check needs to be introduced to compare facility reported costs with PLICS data. Any significant discrepancies should trigger an alert for further investigation.

- **Accuracy of Cost Attribution:** A quality check to ensure the accurate attribution of costs to the respective areas (medicines, consumables, staff, etc.) in line with the PLICS data needs to be implemented.

- **Data Completeness:** We need to ensure that all required financial data fields are populated for each contract, and that these align with the PLICS data. Any missing data should be flagged for follow-up.

This gap analysis points out critical areas in financial data management where the proposed contract management software needs to bring improvements. By bridging these gaps and introducing comprehensive data quality checks, we can ensure that the costs reported by the facilities align accurately with the PLICS data, improving financial transparency and accuracy in the Cluster R1.

Activity Data Gaps

- **Correlation Between Facility Reported Activity and ABM Data**: The current system does not facilitate a robust correlation between the activities reported by the facility and the Activity Based Management (ABM) data. A clear linkage, possibly via the encounter ID within the beneficiary data, is necessary for efficient cross-referencing.
- **Patient Matching Information**: Currently, there are limited fields populated in the ABM and beneficiary data that would allow for fuzzy logic matching of patients (e.g., DOB, gender). More fields need to be populated to ensure accurate patient matching.
- **Beneficiary Data Completeness**: The current beneficiary reports lack critical patient information, making it impossible to verify and validate data effectively.
- **Standardisation of Hospital Acquired Conditions (HAC) Data**: Currently, HAC data is input in free-text comments sections, which hampers detailed and granular analysis. Standardising this data with a coded approach (e.g., the ICD 10 HAC list) would improve clarity and ease of analysis.

Activity Data Quality Checks

- **Data Validation Checks**: Automated checks need to be established for cross-referencing facility reported activity data with ABM data. Discrepancies should prompt alerts for further investigation.
- **Patient Matching Accuracy**: Quality checks should be introduced to ensure the accuracy of patient matching using fuzzy logic. The checks should flag instances where inadequate data is available for accurate matching.
- **Beneficiary Data Verification**: The system needs to implement robust verification checks to ensure the validity of the beneficiary data.
- **Categorisation and Coding of HAC Data**: Quality checks should ensure HAC data is correctly coded as per ICD 10 standards, providing a structured and standardised approach for data analysis.

By identifying and addressing these gaps in activity data management and introducing rigorous data quality checks, we can significantly improve the precision, clarity, and reliability of the activity data reported within Cluster R1.